# TEST REPORT

| | | | |
|---|---|---|---|
| Name | : **Ms. ROOHI SAMA** | Registered on | : 25-Jan-2026 10:17 |
| Age/Gender | : **18 Years / Female** | Collected on | : 25-Jan-2026 10:24 |
| Registration ID | : **260830002637** | Released on | : 25-Jan-2026 16:23 |
| Ref. By | : Dr Self | Printed on | : 25-Jan-2026 16:41 |
| Sample Type | : Serum | Regn Centre | : **Suncity - 83** |

## VITAMIN - B12

| TEST NAME | | RESULT | UNIT | BIOLOGICAL REFERENCE INTERVAL |
|---|---|---|---|---|
| **Vitamin - B12**<br>*Method: Chemiluminescence Immuno Assay (CLIA)* | : | 226 | pg/mL | Deficient : <211<br>Normal : 211 - 911 |

**Interpretation / Comments** :

- Vitamin B12 is essential in DNA synthesis, haematopoiesis and CNS integrity.
- Serum Vitamin B12 level is useful in evaluation of macrocytic anaemia, CNS disorders, alcoholism and malabsorption syndrome and are decreased in megaloblastianemia partial/total gastrectomy, pernicious anemia, peripheral neuropathies, chronic alcoholism, senile dementia, and treated epilepsy.
- High levels of Vitamin B12 may be due to exogenous supplementation. Clinical correlation suggested. Increased levels can be seen in Chronic renal failure, Congestive heart failure, Leukemias, Polycythemia vera, Liver disease etc.
- A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12.
- Vitamin B12 deficiency associated increase in serum / plasma homocysteine levels is an independent risk marker for cardiovascular disease and deep vein thrombosis.
- The most specific test for Vitamin B12 deficiency at the cellular level is the assay for plasma Methyl malonic acid (MMA). If clinical symptoms suggest deficiency, measurement of plasma MMA and plasma homocysteine (more sensitive) should be considered, even if serum Vitamin B12 concentrations are normal or indeterminate.
- Serum Holo-Transcobalamin II level (Active Vitamin B12 level) is a more accurate marker of Vitamin B12 status in body.

MC-2657

**DR.N SUJANA**
**Consultant Biochemist**
Registration No: 50478

* Suggested Clinical Correlation, If Necessary Kindly Discuss with signatory



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## FERRITIN

| TEST NAME | | RESULT | UNIT | BIOLOGICAL REFERENCE INTERVAL |
|---|---|---|---|---|
| **Ferritin** | : | 16.3 | ng/mL | 10 - 291 |

*Method: Chemiluminescence Immuno Assay (CLIA)*

**Interpretation / Comments :**

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- Useful to predict and monitor iron deficiency and differentiate from chronic disease as a cause of anaemia.
- Also useful in identifying iron overload states to plan chelation therapy.
- Ferritin is an acute phase reactant,which means it can be elevated in iron deficiency states due to inflammation and infection.

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## THYROID PROFILE

| TEST NAME | | RESULT | UNIT | BIOLOGICAL REFERENCE INTERVAL |
|---|---|---|---|---|
| **T3 Total**<br>*Method: Chemiluminiscence Immunoassay* | : | 1.29 | ng/mL | 0.86 - 1.92 |
| **T4 Total**<br>*Method: Chemiluminiscence Immunoassay* | : | 9.80 | µg/dL | 5.5 - 11.1 |
| **TSH - Ultrasensitive**<br>*Method: Chemiluminescence Immuno Assay (CLIA)* | : | 0.813 | µIU/mL | Non pregnant : 0.48 - 4.17<br>Pregnant<br>1st Trimester : 0.3 - 4.5<br>2nd Trimester : 0.5 - 4.6<br>3rd Trimester : 0.8 - 5.2 |

Interpretation / Comments :
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- Patient preparation is particularly important for hormone studies, results of which may be markedly affectedby many factors such as stress, position, fasting state, time of the day, preceding diet and drug therapy.
- The levels of T3 helps in the diagnosis of T3 thyrotoxicosis and monitoring the course of hyperthyroidism.T3 is not recommended for diagnosis of hypothyroidism as decreased values have minimal clinical significance.
- Values below the lower limits can be caused by a number of conditions including non-thyroidal illness, acute and chronic stress and hypothyroidism.
- Elevated level of T4 are seen in hyperthyroidism, pregnancy, euthyroid patients with increased serum thyroxine binding globulin.
- Decreased levels are noted in hypothyroidism, hypoproteinemia, euthyroid sick syndrome, decrease in thyroxine binding globulin.
- TSH levels are increased in primary hypothyroidism, insufficient thyroid hormone replacement therapy,Hashimotos thyroiditis, use of amphetamines, dopamine antagonists, iodine containing agents, lithium and iodine induced or deficiency goiter.
- Decreased levels of TSH may be seen in graves disease, toxic multinodular goitre, thyroiditis, excessivetreatment with thyroid hormone replacement and central hypothyroidism.

MC-2657

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---------- End of Report ----------

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